

Name	: Mr. ARUN KUMAR	Age	: 46 Years
Lab No.	: 445629935	Gender	: Male
Ref By	: SELF	Reported	: 2/5/2024 2:26:44PM
Collected	: 1/5/2024 7:22:00AM	Report Status	: Final
A/c Status	: P	Processed at	: LPL-CHENNAI
Collected at	: CHENNAI LAB HOME VISIT		
	KKR ENT Hospital & Research Institute		KKR ENT Hospital & Research Institute
	building,2nd Floor,#6OMR Road Karapakkam		Building, 2nd floor, 6 OMR road,
	CHENNAI 600096		Karapakkam, chennai - 6000096



Test Report

Test Name	Results	Units	Bio. Ref. Interval
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SWASTHFIT COMPLETE PACKAGE

LIVER & KIDNEY PANEL, SERUM

Creatinine (Modified Jaffe,Kinetic)	0.81	mg/dL	0.67 - 1.17
GFR Estimated (CKD EPI Equation 2021)	110	mL/min/1.73m2	>59
GFR Category (KDIGO Guideline 2012)	G1		
Urea (UREASE UV)	22.23	mg/dL	14.9 - 38.5
Urea Nitrogen Blood (UREASE UV)	10.38	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio (Calculated)	13		
Uric Acid (Uricase)	4.54	mg/dL	3.50 - 7.20
AST (SGOT) (IFCC)	22.2	U/L	<50
ALT (SGPT) (IFCC)	16.3	U/L	<50
GGTP (GCNA)	33.5	U/L	<55
Alkaline Phosphatase (ALP) (IFCC AMP-BUFFER)	68.63	U/L	30 - 120
Bilirubin Total (DPD)	1.02	mg/dL	<1.00
Bilirubin Direct (DPD)	0.18	mg/dL	0.00 - 0.30
Bilirubin Indirect (Calculated)	0.84	mg/dL	<1.10
Total Protein (Biuret)	6.69	g/dL	6.40 - 8.30
Albumin (BCG)	4.15	g/dL	3.50 - 5.20
A : G Ratio (Calculated)	1.63		0.90 - 2.00
Globulin(Calculated) (Calculated)	2.54	gm/dL	2.0 - 3.5
Calcium, Total (Arsenazo III)	9.23	mg/dL	8.60 - 10.30



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Test Name	Results	Units	Bio. Ref. Interval
Phosphorus (Molybdate UV)	3.61	mg/dL	2.40 - 4.40
Sodium (Indirect ISE)	139.50	mEq/L	136.00 - 146.00
Potassium (Indirect ISE)	4.80	mEq/L	3.50 - 5.10
Chloride (Indirect ISE)	99.10	mEq/L	101.00 - 109.00



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIPID SCREEN, SERUM			
Cholesterol, Total (CHO-POD)	203.12	mg/dL	<200.00
Triglycerides (GPO-POD)	133.27	mg/dL	<150.00
HDL Cholesterol (CHO-POD)	39.48	mg/dL	>40.00
LDL Cholesterol, Calculated (Calculated)	136.99	mg/dL	<100.00
VLDL Cholesterol, Calculated (Calculated)	26.65	mg/dL	<30.00
Non-HDL Cholesterol (Calculated)	164	mg/dL	<130

Note

- Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL& LDL Cholesterol.
- Additional testing for Apolipoprotein B, hsCRP, Lp(a) & LP-PLA2 should be considered among patients with moderate risk for ASCVD for risk refinement.

Treatment Goals as per Lipid Association of India 2020

RISK CATEGORY	TREATMENT GOAL		CONSIDER THERAPY	
	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)
Extreme Risk Group Category A	<50 (Optional goal ≤30)	<80 (Optional goal ≤60)	≥50	≥80
Extreme Risk Group Category B	≤30	≤60	>30	>60
Very High	<50	<80	≥50	≥80
High	<70	<100	≥70	≥100
Moderate	<100	<130	≥100	≥130
Low	<100	<130	≥130*	≥160*

*In low risk patient, consider therapy after an initial non-pharmacological intervention for at least 3 months



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
GLUCOSE, FASTING (F) (Hexokinase)			
Glucose Fasting	98.45	mg/dL	70.00 - 100.00
VITAMIN B12; CYANOCOBALAMIN (CLIA)			
Vitamin B12; Cyanocobalamin	133.00	pg/mL	180.00 - 914.00

Interpretation

Remarks	Result In pg/mL
Normal	180 - 914
Indeterminate	120 - 180
Deficient	< 120

Notes

1. Interpretation of the result should be considered in relation to clinical circumstances.
2. It is recommended to consider supplementary testing with plasma Methylmalonic acid (MMA) or plasma homocysteine levels to determine biochemical cobalamin deficiency in presence of clinical suspicion of deficiency but indeterminate levels. Homocysteine levels are more sensitive but MMA is more specific
3. False increase in Vitamin B12 levels may be observed in patients with intrinsic factor blocking antibodies, MMA measurement should be considered in such patients
4. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity

VITAMIN D, 25 - HYDROXY, SERUM (CLIA)			
Vitamin D, 25 Hydroxy	22.19	nmol/L	75.00 - 250

Interpretation



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Test Name	Results	Units	Bio. Ref. Interval
LEVEL	REFERENCE RANGE IN nmo1/L	COMMENTS	
Deficient	< 50	High risk for developing bone disease	
Insufficient	50-74	Vitamin D concentration which normalizes Parathyroid hormone concentration	
Sufficient	75-250	Optimal concentration for maximal health benefit	
Potential intoxication	>250	High risk for toxic effects	

Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.
- 25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L.
- It shows seasonal variation, with values being 40-50% lower in winter than in summer.
- Levels vary with age and are increased in pregnancy.
- A new test Vitamin D, Ultrasensitive by LC-MS/MS is also available

THYROID PROFILE,TOTAL, SERUM (CLIA)

T3, Total	1.03	ng/mL	0.70 - 2.04
T4, Total	8.31	µg/dL	5.74 - 13.03
TSH	3.33	µIU/mL	0.34 - 5.60

Note

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50% . hence time of the day has influence on the measured serum TSH concentrations.
- Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
- Unbound fraction (Free,T4 /Free,T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration
- Values <0.03 uIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
IRON STUDIES, SERUM (Spectrophotometry, TPTZ, NITROSO - PSAP)			
Iron	74.00	ug/dL	65.00 - 175.00
Total Iron Binding Capacity (TIBC)	380.00	µg/dL	250.00 - 450.00
Transferrin Saturation	19.47	%	20.00 - 50.00



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
HbA1c (GLYCOSYLATED HEMOGLOBIN), BLOOD (HPLC, NGSP certified)			
HbA1c	6.4	%	4.00 - 5.60
Estimated average glucose (eAG)	137	mg/dL	

Interpretation

HbA1c result is suggestive of at risk for Diabetes (Prediabetes)/ well controlled Diabetes in a known Diabetic

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults >=18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	>= 6.5	<7.0

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.

FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS
Hemoglobin variants,elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g.,recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used.Iron deficiency anemia is associated with higher HbA1c



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Test Name	Results	Units	Bio. Ref. Interval
URINE EXAMINATION, ROUTINE; URINE, R/E (Automated Strip Test, Microscopy)			
Physical			
Colour	Light Yellow		Pale yellow
Specific Gravity	1.010		1.001 - 1.030
pH	6		5.0 - 8.0
Chemical			
Proteins	Negative		Negative
Glucose	Negative		Negative
Ketones	Negative		Negative
Bilirubin	Negative		Negative
Urobilinogen	Negative		Negative
Nitrite	Negative		Negative
Microscopy			
R.B.C.	Negative		0.0 - 2.0 RBC/hpf
Pus Cells	0-1 WBC/HPF		0-5 WBC / hpf
Epithelial Cells	Few		0.0 - 5.0 Epi cells/hpf
Casts	None seen		None seen/Lpf
Crystals	None seen		None seen



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
HEMOGRAM (Photometry,Electrical Impedence,VCS Technology ,Calculated & westergren)			
Hemoglobin	13.98	g/dL	13.00 - 17.00
Packed Cell Volume (PCV)	42.30	%	40.00 - 50.00
RBC Count	4.94	mill/mm3	4.50 - 5.50
MCV	85.70	fL	83.00 - 101.00
MCH	28.30	pg	27.00 - 32.00
MCHC	33.00	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW)	15.50	%	11.60 - 14.00
Total Leukocyte Count (TLC)	6.33	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	51.66	%	40.00 - 80.00
Lymphocytes	36.68	%	20.00 - 40.00
Monocytes	5.86	%	2.00 - 10.00
Eosinophils	5.37	%	1.00 - 6.00
Basophils	0.43	%	<2.00
Absolute Leucocyte Count			
Neutrophils	3.27	thou/mm3	2.00 - 7.00
Lymphocytes	2.32	thou/mm3	1.00 - 3.00
Monocytes	0.37	thou/mm3	0.20 - 1.00
Eosinophils	0.34	thou/mm3	0.02 - 0.50
Basophils	0.03	thou/mm3	0.02 - 0.10
Platelet Count	213	thou/mm3	150.00 - 410.00
Mean Platelet Volume	7.3	fL	6.5 - 12.0
E.S.R.	13	mm/hr	0 - 15



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
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- Note
- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
 - Test conducted on EDTA whole blood



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
PSA (PROSTATE SPECIFIC ANTIGEN), TOTAL, SERUM (CLIA)			
PSA, TOTAL	0.800	ng/mL	<4.00

Note

1. This is a recommended test for detection of prostate cancer along with Digital Rectal Examination (DRE) in males above 50 years of age.
2. False negative / positive results are observed in patients receiving mouse monoclonal antibodies for diagnosis or therapy
3. PSA levels may appear consistently elevated / depressed due to the interference by heterophilic antibodies & nonspecific protein binding
4. Immediate PSA testing following digital rectal examination, ejaculation, prostatic massage, indwelling catheterization, ultrasonography and needle biopsy of prostate is not recommended as they falsely elevate levels
5. PSA values regardless of levels should not be interpreted as absolute evidence of the presence or absence of disease. All values should be correlated with clinical findings and results of other investigations
6. Sites of Non-prostatic PSA production are breast epithelium, salivary glands, peri-urethral & anal glands, cells of male urethra & breast milk
7. Physiological decrease in PSA level by 18% has been observed in hospitalized / sedentary patients either due to supine position or suspended sexual activity
8. The concentration of PSA in a given specimen, determined with assays from different manufacturers, may not be comparable due to differences in assay methods, calibration, and reagent specificity.

Recommended Testing Intervals

- Pre-operatively (Baseline)
- 2-4 days post-operatively
- Prior to discharge from hospital
- Monthly followup if levels are high or show a rising trend

Post Surgery	Frequency of testing
1st year	Every 3 months
2nd year	Every 4 months
3rd year onwards	Every 6 months



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Clinical Use

- An aid in the early detection of Prostate cancer when used in conjunction with Digital rectal examination in males more than 50 years of age and in those with two or more affected first degree relatives.
- Followup and management of Prostate cancer patients
- Detect metastatic or persistent disease in patients following surgical or medical treatment of Prostate cancer

Increased Levels

- Prostate cancer
- Benign Prostatic Hyperplasia
- Prostatitis
- Genitourinary infections

Dr.Jackson Bose
M.D. Pathology
Consultant Pathologist
Dr Lal PathLabs Ltd

Dr M.Sivasankari
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SWASTHFIT COMPLETE PACKAGE			
C-REACTIVE PROTEIN, CARDIO; hsCRP (Immunoturbidimetry)	2.80	mg/L	<1.00

Interpretation

CARDIO CRP IN mg/L	CARDIOVASCULAR RISK
<1	Low
1-3	Average
3-10	High
>10	Persistent elevation may represent Non cardiovascular inflammation

APOLIPOPROTEINS A1 & B, SERUM (Immunoturbidimetry)			
Apolipoprotein (Apo A1)	124	mg/dL	105.00 - 175.00
Apolipoprotein (Apo B)	109	mg/dL	60.00 - 140.00
Apo B / Apo A1 Ratio	0.88		0.35 - 0.98

As per recommendations of National Cholesterol Education Program (NCEP) the clinical significance of results is as follows:

Apolipoprotein B

RESULT IN mg/dL	REMARKS
<23	Abetalipoproteinemia/Hypobetalipoproteinemia
23-45	Hypobetalipoproteinemia
46-135	Normal
>135	Hyperapobetalipoproteinemia/Increased CAD risk

Apo B to A1 Ratio

RATIO	REMARKS
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0.35-0.98	Desirable		
>0.98	Increased CAD risk		



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AMYLASE, SERUM (IFCC)			
Amylase	99.00	U/L	28.00 - 100.00

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Consultant Pathologist

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Chief of Laboratory
Dr Lal PathLabs Ltd

Dr Richa Daljeet Singh
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Dr. Sajith K Satheesh
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Sr. Consultant Pathologist

Dr.Swetha V
MD, Biochemistry
Consultant Biochemist
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-----End of report -----



IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.
(#) Sample drawn from outside source.
If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.
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